INVESTIGATIONS

2/2/21

TO SEND / COLLECT						
	Hb: 9.5	Platelet Counts:	Urine R/M:	O2	✓	CHECK RYLE'S TUBE POSITION BEFORE EACH FEED
	TLC:	DLC:	CXR:	Monitor	✓	
	B. Sugar:	Na/K: 134.3/3.36		ECG		
	Urea:	Creatinine: 0.68	MISC:	Pace Maker		
done	CPK:	MB:		Nebulizer		
	S. Bil T/D/I:			Infusion Pump		
	SGOT / PT / ALP:			B. Sugar		
	S. Proteins/Alb/Glob:			Dialysis		
	PTT/PT/INR:			Ventilator / Bipap		

NURSES NOTES

Sticker

CHARGES

LAB

OTHERS

	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
VENT MODE				RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA
RATE / FIO2				–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–
CPP / PAP				–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–
SSAT				98%	98%	99%	98%	97%	98%	98%	96%	99%	96%	94%	94%	94%	94%	94%	94%	94%	95%	96%	94%	94%	94%	94%	94%
PULSE / HR (rm)				84	82	90	90	90	90	90	80	80	105	84	88	94	84	84	90	84	88	90	90	90	90	90	90
BP				110/70	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80
TEMP				98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F
RR				18/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	18/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min
RBS																											
HEMATOMA																											
DISTAL PULSES																											
INTAKE IV				1000 ml	1000 ml	1000 ml	1000 ml							1000 ml	1000 ml	1000 ml	1000 ml	1000 ml	1000 ml								
INTAKE ORAL				200 ml	200 ml	200 ml	200 ml							200 ml	200 ml	200 ml	200 ml	200 ml	200 ml								
INTAKE R.T.																											
OUTPUT URINE				1000 ml	150					800 ml							800 ml				800 ml						
OUTPUT R.T.																											
OUTPUT STOOL																											
OUTPUT DRAIN																											

ORDERS

2100 ml / 1500 ml

[redacted]

STATUS : CASH / MEDICLAIM / GOVT. PANEL

WARD PATIENT'S DAILY RECORD SHEET

CONSULTANT'S NAME [redacted]

PATIENT'S NAME [redacted]

DAY 5th / 6th

AGE/SEX 20/F ROOM HDW IPD NO. 1457 UHID 22/320

START DATE	S.No.	DESCRIPTION DRUGS	ORDERS DOSE	FREQUENCY	ROUTE	ADMINISTRATION TIME DATE 8/2/26	DATE 9/2/2026
4/2/26	1	INJ - SUPACEF (DS)	1.5gm	TWICE	IV		
	2	INJ - PAN	40mg	TWICE	IV		
	3	INJ - EMEST	4mg	TWICE	IV		
	4	INJ - TRAMADOL	50mg	TWICE	IV		
	5	INJ - OFLOX (DS)	200mg	TWICE	IV		
	6	INJ - PCM	1gm	TWICE	IV		
	7	INJ - PERINORM		TWICE	IV		
	8	INJ - CLEXANE	0.6ml	1D	S/C		
	9						
	10						
	11	T - THYRONORM	50mg	ONCE	P/O		
	12	CAP - BISILAC	1cap	TWICE	P/O		
	13	CAP - ZEDOTT	1cap	ODS	P/O		
	14	CAP - ZERODIET	1cap	ODS	P/O		
	15						
	16	NEB - DUOLIN	1ml	TWICE	P/N		
	17	NEB - BUDECORT	1ml	TWICE	P/N		
	18	SYP - POTRIDE	10ml	TID	P/O		
	19	INJ - HYDROCORT	100mg	STAT	I/V		
	20	INJ - AVIL	1amp	STAT	I/V		
	21	INJ - LASIX	10mg		I/V		
	22						
	23						
	24						
	25						
	26						
	27						
	28						
	29						

[redacted]

INVESTIGATION TO SEND/ COLLECT

DATE 8/2/26 — 03:15 — SE — NIL

DATE 9/2/2026 — CBC sample done at 9am

NIL Arranged.

PROCEDURES

DIET Soft diet high protein diet.

NURSE NOTES

DAY 4th

O2

ECI

INTAKE

BUN

B. SUG

INTAKE PO

OUTPUT

DIARRHOEA

PL. CHECK R.T POSITION BEFORE EVERY FEED

NAME & NATURE OF SURGERY & ANY STAT MEDICATIONS

Medicine over due.

1 unit Blood start at 12:20AM and stop 4:00 AM

Check Fluids, Monitors, I/V Lines, O2 Sp, Hourly Regulary

Total Intake/Output 3500 / 1500 Balance RTA Drain ✓ No

HBsAg/HIV/HCV N/A Drug Allergy N/A Bedsore ✓ No DVT Prophylaxis ✓ No

DATE 8/2/26 IV REGIME & TIMINGS 9/2/2026 DATE

8/2/26	9/2/2026
IV Fluid NS 40ml/hr	IV Fluid NS 50ml/hr

DAY 8/2/26 INVESTIGATION DATE 9/2/2026

[redacted]

DAILY FLUID BALANCE CHART

DAY & DATE

HOUR ENDING	ORAL AMOUNT	ORAL TYPE	INTRAVENOUS PUT UP	INTRAVENOUS GIVEN	URINE	ASPIRATION or VOMIT
7am	100ml	Hot	100ml	often		
	100ml	Tea	100ml	Pen		
9am	200ml	Tea				
	100ml	H2o	100ml	Supracef		
			100ml	Metronidazole		
12pm	250ml	Fruit				
1pm	100ml	H2o			1000ml	
2pm	250ml	Juice	100ml	Rocs		
3pm	50ml	Tea				
5pm	50ml	Soup				
8pm	200ml	Dinner	100ml	Ulcex		
	100ml	H2O	100	Supracef		
			100	Metronidazole	1000ml	
			100	Tramadol		
24 HOURS TOTAL			2200		2000	

9/2/2026 DAY & DATE

HOUR ENDING	ORAL AMOUNT	ORAL TYPE	INTRAVENOUS PUT UP	INTRAVENOUS GIVEN	URINE	ASPIRATION or VOMIT
7AM	100ml	Tea	100ml	Pen		
	100ml	H2O	100ml	Ofloxe		
9AM	[illegible]	[illegible]	[illegible]	[illegible]	[illegible]	
	[illegible]	[illegible]	[illegible]	Pen		
	[illegible]	[illegible]	[illegible]	[illegible]		
10AM	[illegible]	[illegible]	[illegible]	[illegible]		
1PM	[illegible]	[illegible]	[illegible]	[illegible]	[illegible]	
	[illegible]	[illegible]	[illegible]	Pen		
4PM	[illegible]					
	[illegible]	[illegible]				
5PM	[illegible]	[illegible]				
9PM	[illegible]	[illegible]			11PM 200ml	
	[illegible]					
24 HOURS TOTAL	[illegible]				[illegible]	

SISTER ON DUTY

[redacted]

COMA CHART

OBSERVATIONS EVERY HOUR Date: 8/2/2026

TIME	8am	9	10																					
																								5 10 5 10
LEVELS OF CONSCIOUSNESS: Alert & Awake		✓	✓	✓																				✓ ✓
Drowsy & Sl Confused		✓	–	–																				– –
Wdy Drowsy Rousts to Word		–	–	–																				– –
Sem-coma Rousts to pain		–	–	–																				– –
Unresponsive		–	–	–																				– –
PUPILS: Left E.D.R.		2	2	2																				2 2 2
Reactive		+	+	+																				+ + +
Right E.D.R.		2	2	2																				2 2 2
Reactive		+	+	+																				+ + +
BLOOD PRESSURE (mmHg)	240																							
	230																							
	220																							
	210																							
	200																							
	190																							
	180																							
	170																							
	160																							
PULSE (BPM)	150																							
	140																							
	130																							
	120																							
	110																							
	100																							
	90																							
	80																							
	70																							
	60																							
RESPIRATION	50																							
	40																							
	30																							
	20																							
	10																							

[redacted]

REMARKS
Temperature, Movements, Vomiting etc.

[redacted]

STATUS : CASH / MEDICLAIM / GOVT. PANEL

WARD PATIENT'S DAILY RECORD SHEET

[redacted]

DAY 7th

IPD NO. 17454 UHID 25/34

START DATE	S.No.	DESCRIPTION DRUGS	ORDERS DOSE	FREQUENCY	ROUTE	ADMINISTRATION TIME DATE 10/2/26				DATE
	1	INJ SUPACEF 1.5	1GM	TWICE	I/V				6pm	
	2	INJ PAN	40mg	TWICE	I/V	8am		8pm		
	3	INJ EMSET	30mg	THRICE	I/V	8am	4pm	8pm		
	4	INJ TRAMADOL	50mg	THRICE	I/V	8am			8pm	
	5	INJ OFLOX	200mg	THRICE	I/V	8am		8pm		
	6	INJ PCM	1GM	THRICE	I/V	8am	4pm	8pm		
	7	INJ PERINORM		THRICE	I/V	8am	4pm		10pm	
	8	INJ CLEXANE	0.4ml		S/C				10pm	
	9									
	10	TAB THYRONORM	50mcg	ONCE	P/O	8am				
	11	CAP BIFILAX	1cap	THRICE	P/O	8am			10pm	
	12	CAP ZEPOTS	1cap	SOS	P/O					
	13	CAP ZEPODIUM	1cap	SOS	P/O					
	14									
	15	NEB-DUOLIN	1vial	THRICE	P/N	8am	4pm	8pm		
	16	NEB BUDECORT	1vial	TWICE	P/N	8am	8pm	8pm		
	17	S/C FOTBICR	1ml	THRICE	P/N	8am	4pm	8pm		
	18									
	19									
	20									
	21									
	22									
	23									
	24									
	25									
	26									
	27									
	28									
	29									

[redacted]

DATE 10/2/26	IV REGIME & TIMINGS	DATE
IV	RINGS NS 100ml/hr	

INVESTIGATION TO SEND/ COLLECT	DATE
SAMPLE 10/2/26	
[redacted]	

PROCEDURES

DIET Soft Diet
High Protein diet

NURSE NOTES			
DATE			
O2			
ECG			
NEB			
BI/PU			
B. SUGAR			
INFUSION PUMP			
DIALYSIS			
DIAGNOSIS			

PL. CHECK R.T POSITION BEFORE EVERY FEED

NAME & NATURE OF SURGERY & ANY STAT MEDICATIONS

Check I/V Fluids, Monitors, I/V Lines, O2 ½ Hourly Regularly

Total Inside/Outside | Balance | RTA Yes | Drain No

HISBAg/HIV/HCV N.K. | Drug Allergy N.K. | Bedsore | DVT Prophylaxis

DATE	10/2/26	INVESTIGATION	DATE

[redacted]

DAILY FLUID BALANCE CHART

DAY & DATE

10/2/25

HOUR ENDING	ORAL AMOUNT	ORAL TYPE	INTRAVENOUS PUT UP	INTRAVENOUS GIVEN	URINE	ASPIRATION OR VOMIT	
7 am	100ml	Tea			900		
	100ml	H2O					
9am	Porridge	B.P					
	50ml	Tea	100ml	Sugared			
10am	100	H2O	100ml	Dextrose			
24 HOURS TOTAL							

TEMPERATURE

PULSE

RESPIRATION

SISTER ON DUTY		
	Dr.	Dr.
M	[redacted]	
A/N		
N		

DAY & DATE

HOUR ENDING	ORAL AMOUNT	ORAL TYPE	INTRAVENOUS PUT UP	INTRAVENOUS GIVEN	URINE	ASPIRATION OR VOMIT	
24 HOURS TOTAL							

COMA CHART

OBSERVATIONS EVERY ____ HOUR

TIME	10/2/25																							
LEVELS OF CONSCIOUSNESS: Alert & Awake	✓																							
Drowsy & Sl Confused	✓																							
Very Drowsy Reacts to voice	✓																							
Semi-comatose Reacts to pain	✓																							
Unresponsive	✓																							
PUPILS: Size R	1																							
Reaction	2																							
Size L	3																							
Reaction	4																							
BLOOD PRESSURE (GREEN)	260																							
	250																							
	240																							
	230																							
	220																							
	210																							
	200																							
	190																							
	180																							
	170																							
	160																							
	150																							
PULSE (RED)	140																							
	130																							
	120																							
	110																							
	100																							
	90																							
	80																							
	70																							
	60																							
RESPIRATION	50																							
	40																							
	30																							
	20																							
	10																							
REMARKS Temperature, Movements, Vomiting etc																								

[redacted]

[redacted]

2

ICU-10

INFORMED CONSENT FOR SURGERY / PROCEDURE

INFORMED CONSENT FOR Rt TKR

I, [redacted] ☐ the patient, or ☐ (representative of patient .....), (please tick the correct option above and below).

I have read and have been explained this consent form in English (name of language) which fully I understand and understood the information provided about ..... (full name of operation/procedure) given below in this consent form.

B.2. description of operation/procedure.....

Intended benefits

1. Relief from diagnosed illness
2. _____
3. _____
4. _____
5. _____
6. _____

I understand that all procedures carry risks. The potential risks and complications from this procedure are:

1. Bleeding
2. Infection
3. _____
4. _____
5. _____
6. _____
7. _____
8. _____
9. _____
10. _____

I have been explained the implications of not undergoing this procedure and the alternative metho[redacted]f treatment like

1. _____
2. _____
3. _____
4. _____

I am now aware of the intended benefits, possible risks & complications, and ava[redacted]e the said operation/ procedure. I am also aware that results of any operation/procedure can va[redacted]nt; and I declare that no guarantees have been made to me regarding success of this operation/procedur[redacted] majority of patients have an uneventful operation and recovery, few cases may be associated with complicatio[redacted] the common risks and complications associated with this operation/procedure as listed in Part B, and understand that it i[redacted]ssible to list all possible risks and complications of any operation/procedure.

I also understand that sometimes a planned operation/procedure may to be postponed or cancelled if patient's clinical condition worsens or due to any unforeseen technical reason. I am also aware that I can withdraw my consent at any point of time at my own risk and consequence, by submitting the withdrawal in writing.

Initials of Patient / Patient's Representative (only if Patient is a minor or unable to give consent)_____

I understand that if medical exigencies demand, further or alternative operation/procedure measures may need to be carried out, and in such case there may be difference in the planned and actual operation/procedure. I am aware that I may require administration of blood and/or blood products during or after the operation/procedure as found necessary by the doctor (for which a separate consent shall be obtained).

I am now also aware that during the course of this operation/procedure the doctor will be assisted by medical and paramedical team, and that the doctor may seek consultation/assistance from relevant specialists if the need arises.

I agree to observing, photography (still/video/television) of the procedure (including relevant portions of my body) including my diagnosis/report (pathology, radiology, etc), for academic/medical/medico-legal purposes, provided my identity is not relevant by such acts. I also agree to my clinical details being shared for scientific publications if my identity is not disclosed.

I am also aware of the expected course after the operation/procedure and the care to be provided, and understand that sometimes admission to an intensive care unit and/or extension of duration of hospitalization may be required and/or there may be requirement of extra medicines or treatments, therapy leading to increase in the treatment expenses, depending upon the body's response to the treatment/procedure.

I authorize the hospitals for disposal of any tissue or body part that may be removed from my body in the appropriate manner during and for the purpose of conducting this operation/procedure.

I declare that I have received & fully understood the information provided in this consent form, that I have been given an opportu[redacted] to ask questions relating to my aliment, the operation/procedure being performed, its risks, consequences, alternative, potential complications and intended benefits and recovery, and that all my question have been answered to my entire satisfaction and there are no misconceptions or false hopes in my mind. I further declare that all the fields (of this form) requiring insertion or completion were filled in my presence at the time of my signing this form.

For the above mentioned operation(s)/procedures(s) that I have been made aware of, I give my consent voluntarily to Dr. _____ [redacted] _____(name of the doctor performing the operation/procedure) or carrying out the said operation/procedure on myself or my above named patient being fully aware of the nature, potential risks and complications, intended benefits and possible alternatives.

I, the above named Patient/ named patient's representatives, do further hereby declare that I am above 18 years of age as on the date of signing this form, mentally sound and am giving consent without any fear, theat of false misconception.

	Signatu[redacted]	Name	Date	Time
Patient [redacted]	[redacted]	[redacted]	5/2/25	8 Am.
Relatives/Guardian	[redacted]	Grandson (Write name & relationship with Patient)	5/2/25	8 Am.
Witness				

* Right Hand for Males & Left Hand for Females

only if patient is a minor or unable to give consent

I, the undersigned doctor, have explained the [redacted]al risks and complications, intended benefits, expected post-procedure course, and possible alternative[redacted] operation/procedure, to the patient/patient's representative. I am confident that he/she has understood t[redacted]ully as described in this document.

Consent obtained by:

Dr. [redacted] _____ De[redacted]n & Dept. DOD _____ Signature: [redacted]

Date:_____ Time:_____

Does this patient/procedure require an additional/ HIGH RISK consent? YES NO [redacted]

Doctor performing the procedure : Dr._____ Dept._____ Signature:_____

For Internal/Hospital use only

A9H/IPD/2025-26/INFORM-CONSENT

INFORMED CONSENT FOR ANESTHESIA

I, [redacted] ☐ the patient, or ☐ (representative of patient .....), have (please tick the correct option above and below).

I have read and have been explained this consent form in English (name of language) which I fully understand, that I would require 'Anesthesiology Services' for getting ..... (Operation/Procedure) performed.

It has been explained to me that all forms of anesthesia involve some risks, and though rare, anesthesia related complications like drugs reactions, infections, bleeding, blood clots, loss of sensation, loss of limb function, paralysis, stroke, brain damage, heart attack and death can occur, I understand that these risks apply to all forms of anesthesia and that additional risks specific for the type anesthesia as applicable in my case and marked below have been explained to me as well. I understand that the type(s) for anesthesia service checked below will be used for my procedure and that the anesthetic technique to be used is d[redacted]y facto including my physical condition, the type of procedure my doctor is to perform, anesthe[redacted]quirement of the procedure and accommodating my preference to the extent feasible. It [redacted]mes, an anesthesia technique particularly involving the use of local anesthetics, with or [redacted]y fail to produce desired effect therefore me have to so supplemented with another technique including adminis[redacted] general anesthesia.

• General Anestheisa	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Expected result	Total unconscious state, possible placement of a tube into the windpipe.
	Major Risks	Mouth or threat pain, hoarseness, injury to mouth or teeth, awareness under anesthesia, injury to blood vessels, aspiration, pneumonia, convulsions, Pneumothorax.
• Spinal or Epidural Analgesia • With sedation • Without sedation	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Expected result	Total unconscious state, possible placement of a tube into the windpipe.
	Major Risks	Mouth or threat pain, hoarseness, injury to mouth or teeth, awareness under anesthesia, injury to blood vessels, aspiration, pneumonia, convulsions, Pneumothorax.
• Nerve Block • With Sedation • Without Sedation	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Expected result	Total unconscious state, possible placement of a tube into the windpipe.
	Major Risks	Mouth or threat pain, hoarseness, injury to mouth or teeth, awareness under anesthesia, injury to blood vessels, aspiration, pneumonia, convulsions, Pneumothorax.
• Intravenous Regional Anesthesia • With Sedation • Without Sedation	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Expected result	Total unconscious state, possible placement of a tube into the windpipe.
	Major Risks	Mouth or threat pain, hoarseness, injury to mouth or teeth, awareness under anesthesia, injury to blood vessels, aspiration, pneumonia, convulsions, Pneumothorax.

• Monitored Anesthesia Care with sedation	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes producing a semiconscious state.
	Expected result	Reduced anxiety and pain, partal or total amnesia
	Major Risks	An unconscious state, depressed breathing (requiring ventilation) injury to blow vessels
• Monitored Anesthesia Care without sedation	Technique	None
	Expected result	Measurement of vital signs, availability of anesthesia provider for interventions as deemed fit.
	Major Risks	Awareness, anxiety and/ or procedure related discomfort.
• Rectal/Vaginal/Derma Medication	Technique	Medicine introduced through Rectum/Vagina/Skin to produce loss of sensation
	Expected result	Producing loss of sedation in patients who can't take oral/IV/IM medication or in whom injections are to be avoided.
	Major Risks	Discomfort
• Vascular Cannulation	Technique	Cannulation of artery or large veins
	Expected result	Measurement of invasive arterial pressures/fill up pressures.
	Major Risks	Haematoma, bleeding, ischaemic related complications.

BY SIGNING THIS FORM, I GIVE MY FULL, FREE, VOLUNTARY CONSENT WITHOUT ANY RESERVATION TO DR.

[redacted].....(name of doctor administering anesthesia) or his/her associated, for administering anesthesia to [ ] MYSELF or [ ] MY PATIENT..[redacted].....(name of patient), being fully aware of the nature, risks and complications.

Signed on...../...../ at .....Am/Pm

	Sig[redacted]	Name
Patient [redacted]	[redacted]	[redacted]
Relatives/Guardian	[redacted]	Friend Son (Write name & relationship with Patient)
Witness	[redacted]	
Interpreter (If applicable)	[redacted]	

only if patient is a minor or unable to give consent

I, the undersigned Doctor, have e[redacted]re, potential risks and complications, expected benefits, expected post-anesthesia recovery, and po[redacted]ive to the patient/patient representative. I am confident that he/she have understood the information pr[redacted]n this documents.

Consent obtained by

Dr. [redacted]..... Designation & Dept. ....DoD Anesthesia..... Signature..[redacted]

*(being person administering/Anesthsia or member of his/her team)

[redacted]

Patient Name:.....UHID No.:.....IPD No.:.....

Age/Sex:..... Bed No:..... D.O.A.....

Proposed Date & Time of Surgery: Weight: Height :-

PRE ANESTHETIC CHECKUP & ANESTHESIA RECORD FORM

HISTORY

1. Present History-cough/breathlessness/ chest Pain, Palpitations of Blackouts/ Convulsions of Fits/ Bruising or bleeding problems/ Any other serious illnesses.
2. Past History - T.B / Hypertension/ Diabetes/ Heart attack or heart failure / jaundice.
3. Drug History any medicine or drugs intake (including inhalers, eye drops, creams. Or herbal remedies allergy to any drugs or materials/ women-pregnancy/ pill or HRT
4. Personal History- smoke/ alcohol/ other addition
5. Past surgery / Anesthesia History any previous operations or anesthetics/ any problems with anesthetics with You or family.

[redacted] 2 yrs ago

1. [redacted]
2. [redacted]
3. .....
4. .....

EXAMINATION

Pallor —	Pulse 78	CVS	Oral Hygiene
Icterus / Jaundice	B. P. 130/78	RS	Dentition- false, capped or crowned
Cyanosis —	Respiration NVBS (+)	ABD NAD	Teeth Edentulous
Pedal Oedema —	Temperature (N)	CNS	Mouth Opening >2 finger
Lymphadenopathy —		SPINE	Neck
		FUNDUS EXAM	Airway

Plan of Anaesthesia:- ↓ CSE

INVESTIGATIONS – WNL

HB	B. SUGAR-F	Urine Exam ☐	X-Ray
PCV	PP	LFT:	ECG
TLC	R		Echo
DLC	S. Na+		Hb5Ag
ESR	S. K+		HIV
AEC	S. Ca ++		
PLATELETS	B. Urea:		
BT: ☐ CT: ☐ PT: ☐	S. Creatinine:		

USGASA Grading- I / (II) / III / IV / V / VI / E

Advice : FIT/UNFIT For Surgery under Anesthesia

Any Investigation required :

Treatment required

Pre Anesthetic Orders:

1. Nil orally From: 9 PM
2. Written consent to be taken
3. Arrange unit of crossed matched blood 6 PRBC
4. Premedications Night before surgery
5. Shift Patient to OT By .....

[redacted]

[redacted]

Name & Signature of Anesthetist

Time:_____(AM/PM)

[redacted]

NAME: AGE/SEX:

BED NO: UHID NO.:

TREATING CONSULTANT:

ANESTHESIA MONITORING SHEET

PER - Operative Records

Surgical Procedure:-

Anesthesiologist: [redacted]

Surgeon:-

Anesthesia Time 6:40 pm | Operation Time 7:05 pm | Completion Time 9:10 pm

Anesthesia Technique
GA
Regional Anesthesia
Local Anesthesia
MAC
Inducing Agents-
Muscle Relaxants-
Anesthetic Agents-

CSE
Epidural - APP | Epidural catheter placed at L3-L4 | Midline | Sitting | 18G Tuohy needle | Test dose given
SAB - APP | SAB at L4-L5 | Midline | Sitting | 26G Quincke | 0.5% 3.4ml (4ml) Bupivacaine + 25mcg fentanyl.

Immediate Pre-anesthesia Revaluation:-

ECG:
NIBP:
Temp:
ETCO2:

SPO2:
IBP:
CVP:
NOP: 7:40 pm 8:40 pm 9:10 pm

CVS:
RS:
CNS:
P/A:

AIRWAY:

[redacted]

Intra op IV Fluids NS → NS → NS
Blood
Blood Loss 400 ml
Urine Output 200 ml

Reversal Neostigmine
Atropine Glycopyrrolate

Time: AM/PM
Drugs -

Recovery Score

Post Operative Condition (Recovery Room)

Pulse 79
B.P. 110/70
Temperature
RR
Color
Sensorium
SpO2 98% in Room

Shift to Ward/ICU
NPOTI
Monitor
Analgesia Section
IV fluids

Name & Signature with Employee ID of Nurse
Time [redacted] (AM/PM) 6:30 pm

Name & Signature of Anesthetist
Time [redacted] (AM/PM) 6:40 pm

Name & Signature of Chief Surgeon
Time 9:10 [redacted] (AM/PM)

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[redacted]

[redacted]

[redacted]

F/SEX: .....

2: .....

SURGICAL SAFETY CHECKLIST (WHO)

BEFORE INDUCTION OF ANAESTHESIA
(with at least nurse and anaesthesia)

Has the patient confirmed his/her identity, sit, procedure and consent?
- [x] Yes

Is the Site Marked?
- [x] Yes
- [ ] Not Applicable

Is the Anaesthesia Machine & Medication Check complete?
- [x] Yes

Is the Pulse Oximeter On the Patient Functioning?
- [ ] No
- [x] Yes

Does patient have a?

Known Allergy
- [x] No
- [ ] Yes

Difficult Airway/aspiration Risk
- [ ] No
- [x] Yes

Risk Of > 500ml Blood Loss (7ml/kg In Children)
- [ ] No
- [x] Yes

BEFORE SKIN INCISION
(with nurse, anaesthetist and surgeon)

- [x] Confirm All Team Members Have Introduced Themselves By Name And Role?
- [x] Confirm the patient's name, procedure and where the incision will be made?

Has Antibiotic Prophylaxis Been Given Within The Last 60 Minutes?
- [ ] No
- [x] Yes

Anticipated Critical Events

To Surgeon
- What Are The Critical Steps?
- How long will the case take?
- What is the anticipated blood loss?

To Anaesthetist
- [ ] Are there any patient specific concerns?

To Nursing Team
- [x] Has Sterility Been Confirmed?
- [ ] Are There Equipment Issues?

Is Essential Imaging Displayed?
- [ ] No
- [x] Yes

BEFORE PATIENT LEAVES OPERATING ROOM
(with nurse, anaesthetist and surgeon)

Nurse Verbally Conforms
- [x] Name of the procedure
- [x] Completion of instrument, sponge and needle
- [ ] Specimen labelling
- [ ] Whether there are any equipment problems to be addressed?

To Surgeon, Anaesthetist & Nurse
- [ ] What are the key concerns for recovery and management of this patient?

[redacted]

[redacted]

Name & Sign of Nurse
6:40 pm

Name & Sign of Surgeon
9:10pm

Name & S[redacted] 6:40pm

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A9H/IPD/2025-26/SURGICAL-SEFETY-CHECKLIST

[redacted]

[redacted]

[redacted]

[redacted]

15/2/26

IPD No. 1454

OPERATION THEATRE NOTES:

Surgeon In-charge: [redacted] Anaesthetist: [redacted]

Assistant Surgeon: OT Technician: [redacted]

OT Staff: [redacted] Type of Anaesthesia: ✓ CSE

Pre-Operative Diagnosis: Osteoarthritis Bilateral knee

Post-Operative Diagnosis:

Procedure Name: Total knee Replacement Right side

Operation Started at: 7:00 pm Operation Finished at: 9 pm Duration: 2 hrs

Sponge Count: 20 Whom: Dhiraj

Operative Notes:

After adequate effect of the anaesthesia painting and draping of the part was done. Tourniquet inflated. Medial parapatellar approach used to expose the knee joint using midvastus. Tibial and femoral cuts taken. Trial implant found to be satisfactory. B Femur, 1 Tibia. Insert 9 mm size 10.5 x PCL. Final implant placed. Closure done in layers. Drain. Patient shifted to the post operative ward in stable vital condition.

Organ Explained: NA

Specimen Sent for histopathology (if any): Nil

Immediate post-operative condition:

[redacted]

Surgeon's Signature: [redacted]

Date & Time: 15/2/26 9:10 (am/pm)

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A9H/IPD/2025-26/OT NOTES

Post Operative Notes:

- NPO for 6 hrs
- Supine position
- Monitor vitals
- O2 by face mask 4L/min
- RL/NS - 60 ml/hr
- Epidural infusion
  - 20 ml 0.1% Ropivacaine + 2 ml Fentanyl + 0.8 ml NS
  - 4-6 ml/hr
- Transfuse 10 PRBC

Mean

[redacted]

Signature:

Name: Dr. Sourabh Sharma, Dr. Geetika

Time:

- Carry normally

Recovery Room Notes:

- Oxygen @ 4L/min for 12 hrs. — X Ray B/L lung OK
- Transfuse 10 PRBC
- IV fluid NS @ 80 ml/hr
- Inj Sulperazone 1.5 gm IV BD
- Inj Ofloxacin 200 mg IV BD
- Inj Pantop 40 mg IV BD
- Inj Emeset 4 mg IV TDS
- Inj Tramadol 50 mg IV TDS
- Inj Paracetamol 1 gm IV TDS
- Monitor vitals
- Inj Clexane 0.6 ml HS
- DVT Pump — Limb Elevation + Active ROM

Signature: [redacted]

Name:

Time: Dr. Amit Kumar Garg

ID : 26320

5/2/26

LOT C2511003 | REF MK-MLPSXE109

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POSTERIOR STABILIZED
TIBIAL ARTICULAR SURFACE
SIZE B1-2.09MM HEIGHT

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IN 2025-11-16 | 2030-11-15

Meril Orthopedics | MD | CE 2460 | STERILE EO
Meril Healthcare Pvt. Ltd.
M.L.NO. MFG/MD/2018/000063
Rev.02
MH/AW/KS/010

LOT A2504222 | REF MK-UPPSRB00

DESTIKNEE™ | Knee System
POSTERIOR STABILIZED
FEMORAL COMPONENT
SIZE B RIGHT

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(17)301113
(21)017
(10)A2504222

IN 2025-11-14 | 2030-11-13

Meril Orthopedics | MD | CE 2460 | STERILE EO
Meril Healthcare Pvt. Ltd.
M.L.NO. MFG/MD/2018/000063
Rev.02
MH/AW/KS/010

LOT U29162714 | REF RTUUX100-RK

FREEDOM® TOTAL KNEE SYSTEM
STEMMED TIBIAL BASE PLATE
SIZE 1

(01)00811767024242
(10)U29162714
(21)251001
(17)300910
(240)RTUUX100-RK

2025-10-01 | 2030-09-30

Maxx Orthopedics
2460 General Armistead Ave.
Suite 100, Norristown
PA 19403, U.S.A.
maxx Orthopedics | CE 2460 | STERILE EO
Rev.03

LOT U29202707 | REF RSUU3105-RK

FREEDOM® TOTAL KNEE SYSTEM
STEM EXTENSION
SIZE 10.5MM x 100MM

(01)00811767024426
(10)U29202707
(11)250901
(17)300831
(240)RSUU3105-RK

2025-09-01 | 2030-08-31

Maxx Orthopedics
2460 General Armistead Ave.
Suite 100, Norristown
PA 19403, U.S.A.
maxx Orthopedics | CE 2460 | STERILE R
Rev.03

Attach to patient's record
68057851 PALACOS® R
LOT 77421505
2030-08-31
Heraeus Medical GmbH, 61273 Wehrheim, Germany

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI25006**	Registration	: 10/Feb/2026 11:34AM
UHID/MR No	: RCI.0000022214	Collected	: 10/Feb/2026 11:34AM
Patient Name	: [redacted] **IPD**	Received	: 10/Feb/2026 11:35AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 10/Feb/2026 12:56PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10083247

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
LIVER FUNCTION TEST				
Sample Type : SERUM				
TOTAL BILIRUBIN	1.07	mg/dl	0.1-1.2	Diazotized, Sulfanilic
CONJUGATED (D. Bilirubin)	**0.32**	mg/dl	<0.3	Jendrassik & Groff
UNCONJUGATED (I.D. Bilirubin)	0.75			
SGOT	**61**	U/L	<31	Enzymatic,IFFC
S.G.P.T	28	U/L	0-34	Enzymatic,IFFC
ALKALINE PHOSPHATASE	94	U/L	30-120	Kinetic, IFCC
TOTAL PROTEINS	**5.91**	gm/dl	6.6-8.3	Biuret
ALBUMIN	**2.97**	gm/dl	3.4-4.8	BCG
GLOBULIN	2.94	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.01		1.0-2.0	Calculated
GGT	**53**	U/L	<38	Colorimetric Method

Note- 1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.

2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic Hepatitis, Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.

3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

Clinical correlation is essential for final diagnosis | If test result are unsatisfactory please contact personally or on phone. This report is for perusal of doctors only.
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[redacted]

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Visit ID	: **RCI24936**
UHID/MR No	: RCI.0000022144
Patient Name	: [redacted] **IPD**
Age/Gender	: 75 Y 1 M 0 D /F
Ref Doctor	: [redacted]
Client Name	: [redacted]
Client Add	:

Registration	: 09/Feb/2026 10:16AM
Collected	: 09/Feb/2026 10:17AM
Received	: 09/Feb/2026 10:17AM
Reported	: 09/Feb/2026 10:37AM
Status	: Final Report
Client Code	: 315
Barcode No	: 10083177

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
CBC				
Sample Type : WHOLE BLOOD EDTA				
HEMOGLOBIN (HB)	**10.2**	g/dl	11.0-15.0	CYANMETHEMOGLOBIN
TOTAL LEUCOCYTE COUNT (TLC)	7.34	10^3/uL	4.0-10.0	Flow cytometry
NEUTROPHIL	70	%	40-80	
LYMPHOCYTE	**16**	%	20-40	
MONOCYTE	09	%	2-10	
EOSINOPHIL	04	%	1-6	
BASOPHIL	01	%	<2	
ABSOLUTE NEUTROPHIL COUNT	5.13	10^3/uL	2.0-7.0	Automated Calculated
ABSOLUTE LYMPHOCYTE COUNT	1.17	10^3/uL	1.0-3.0	Automated Calculated
ABSOLUTE MONOCYTE COUNT	0.66	10^3/uL	0.2-1.0	Automated Calculated
ABSOLUTE EOSINOPHIL COUNT	0.29	10^3/uL	0.02-0.5	Automated Calculated
ABSOLUTE BASOPHIL COUNT	0.07	10^3/uL	0.02-0.1	Automated Calculated
RBC COUNT(RED BLOOD CELL COUNT)	3.84	million/µL	3.8-4.8	Optical Flowcytometry
PCV/HAEMATOCRIT	**30.4**	%	36-46	RBC pulse height detection
MCV	**79.2**	fL	80-100	Automated/Calculated
MCH	26.6	pg	26-34	Automated/Calculated
MCHC	33.6	g/dl	32-36	Automated/Calculated
RDW-CV	12.7	%	11.6-14.0	Automated/Calculated
RDW-SD	41.6	fL	39-46	Calculated
PCT	0.247	%	0.108-0.282	
MPV	9.6	fL	7.0-12.0	Calculated
PDW	12.1	fL	9-17	Calculated
PLATELET COUNT	257	10^3/uL	150-410	Optical Flowcytometry

[redacted]

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[redacted]

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IP NO: 1454

Visit ID	: **RCI24847**	Registration	: 07/Feb/2026 07:41AM
UHID/MR No	: RCI.0000022055	Collected	: 07/Feb/2026 07:52AM
Patient Name	: [redacted] **IPD**	Received	: 07/Feb/2026 07:54AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 07/Feb/2026 09:26AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	: [redacted]	Barcode No	: 10083088

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
HAEMOGLOBIN (HB)				
Sample Type : Whole Blood				
HAEMOGLOBIN (HB)	9.5	g/dl	11.0-15.0	CYANMETHEMOGLOBIN

NOTE:
Hemoglobin screening helps to diagnose conditions that affect RBCs such as anemia or polycythemia.

[redacted]

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IP NO: 1454

Visit ID	: **RCI24847**	Registration	: 07/Feb/2026 07:41AM
UHID/MR No	: RCI.0000022055	Collected	: 07/Feb/2026 07:52AM
Patient Name	: [redacted] **IPD**	Received	: 07/Feb/2026 07:54AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 07/Feb/2026 09:26AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	: [redacted]	Barcode No	: 10083088

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
SERUM SODIUM				
Sample Type : Serum				
SERUM SODIUM	134.3	mmol/L	133-147	ISE

Increased In:
Dehydration, Vomiting or diarrhea, Burns or excessive sweating, Drugs: infusion hypertonic saline, Diabetes insipidus (DI), Post-traumatic, Renal failure and other renal causes

Decreased In:
Nephrotic syndrome, cirrhosis, pancreatitis, bowel obstruction, Blood loss, hypothyroidism, Adrenal insufficiency.

SERUM POTASSIUM				
Sample Type : Serum				
SERUM POTASSIUM	**3.36**	mmol/l	3.50-5.20	ISE

Increased In:
Vomiting, Diarrhea, Excessive sweating, Extensive burns, Renal failure associated with dehydration, obstruction, trauma, Renal toxicity, Addison disease, Drugs (e.g., NSAIDs, ACE inhibitors), Pseudohypoaldosteronism, use of hypertonic solutions (e.g., saline, mannitol), Intravascular hemolysis (e.g., transfusion reaction, hemolytic anemia), Rapid cellular release (e.g., crush injury, chemotherapy for leukemia or lymphoma, burns, major surgery)

Decreased In:
Uncontrolled diabetes, Renal tubular acidosis, Cushing syndrome, Malignant hypertension, Diuretics, High-dose glucocorticoids.

[redacted]

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[redacted]

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24847**	Registration	: 07/Feb/2026 07:41AM
UHID/MR No	: RCI.0000022055	Collected	: 07/Feb/2026 07:52AM
Patient Name	: [redacted] **IPD**	Received	: 07/Feb/2026 07:54AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 07/Feb/2026 09:26AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 319
Client Add	: [redacted]	Barcode No	: 10063088

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
SERUM CREATININE				
Sample Type : Serum				
SERUM CREATININE	0.68	mg/dL	0.5-0.9	Jaffe

Increased In:

- Diet: digestion of creatinine (meat meal), Muscle disease, gigantism, acromegaly.
- Impaired Kidney function.

Decreased In:

- Pregnancy. Normal value is 0.4-0.8 mg/dL. A value <0.4 mg/dL is abnormal and should alert the clinician to further diagnostic evaluation.
- Creatinine secretion is inhibited by certain drugs (e.g., cimetidine, trimethoprim).

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]